

APPEAL EVIDENCE FACE SHEET

Record Label:	A	Scenario:	Failed Conservative Care With Prog...
Patient:	Jane Doe	DOB:	04/15/1982
Age:	41	Member ID:	DXZ8887304
Claim ID:	9872304	Payer:	HealthPlus Insurance
Provider:	Mercy Hospital	Attending:	Dr. Emily Roberts
Appeal Date:	08/15/2023	Appeal ID:	A-1001
Service Requested:	CPT 72148 MRI lumbar spine without...		

PACKET INTENT

Synthetic appeal-support packet prepared for demonstrations. This record compiles outpatient and consult documentation supporting medical necessity for denied lumbar MRI authorization.

DIAGNOSES UNDER REVIEW

- M54.16 Lumbar radiculopathy, lumbar region
- M51.26 Other intervertebral disc displacement, lumbar region

FAX COVER SHEET (IMPORTED)

TRANSMISSION DETAIL

- Date: 08/15/2023
- ATTN: HealthPlus Insurance
- Appeal request for Jane Doe (DOB 04/15/1982)
- Claim ID: 9872304
- Service requested: CPT 72148 MRI lumbar spine without contrast
- Originating facility: Mercy Hospital

APPEAL REQUEST DETAILS

Patient:	Jane Doe	Member ID:	DXZ8887304
Payer:	HealthPlus Insurance	Provider:	Dr. Emily Roberts, Mercy Hospital
Appeal Submission:	08/15/2023	Procedure:	Lumbar MRI
CPT:	72148	Diagnosis 1:	M54.16 Lumbar radiculopathy, lumbar...
Diagnosis 2:	M51.26 Other intervertebral disc di...		

DETAILED CLINICAL NOTES

- 3-month history of severe low-back pain radiating to left leg.
- Neurologic symptoms include numbness/tingling and progressive weakness.
- Conservative care completed without sustained improvement.
- Latest clinical findings suggest progressive disc pathology with nerve root involvement.

APPEAL LETTER TO MEDICAL DIRECTOR

Date: 08/15/2023

Recipient: Medical Director, HealthPlus Insurance

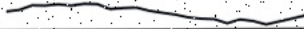
Subject: Appeal for authorization of lumbar MRI for Jane Doe

APPEAL NARRATIVE

Mercy Hospital requests reconsideration of the denied lumbar MRI. Recent evaluations and specialist documentation demonstrate failed conservative treatment, progressive neurologic findings, and the need for advanced imaging to guide definitive management.

REQUESTED ACTION

- Reverse initial denial for CPT 72148 (lumbar MRI without contrast).
- Authorize timely imaging due to progressive deficits and worsening function.


Dr. Emily Roberts

Appeal Author | Signed: 08/15/2023 09:14

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CHRONOLOGY OF RECENT CLINICAL EVENTS

Date	Source	Event	Type	Outcome
05/22/2023	Mercy PCP	Initial low back pain + left leg radiation	Outpatient	NSAID + PT referral
06/05/2023	Mercy Imaging	Lumbar CT (soft tissue limitation)	Diagnostic	Possible disc protrusi...
06/26/2023	Mercy PT	PT start: severe disability ODI 52%	Therapy	2x/week x 6 weeks
07/18/2023	Spine Clinic	Persistent radicular pain despite therapy	Consult	Injection + continue PT
08/03/2023	Neurology	Progressive weakness and reflex changes	Consult	Urgent MRI advised
08/07/2023	Mercy PT	Discharge: ODI 56%, strength down to 3+...	Therapy	Failed conservative c...

REVIEWER NOTES

- Timeline aligns with appeal packet and demonstrates progression despite conservative management.

RECENT OFFICE NOTE - MERCY PRIMARY CARE

Date: 07/10/2023

Provider: Dr. Emily Roberts

Visit Type: Outpatient follow-up

HISTORY / SYMPTOMS

- Three-month history of low back pain radiating to left posterior leg with nightly sleep disruption.
- Symptoms now limit sitting to less than 15 minutes and impair work participation.
- Home exercise program compliance confirmed; no sustained symptom control.

ASSESSMENT

- Lumbar radiculopathy symptoms remain high severity.
- Functional tolerance remains poor despite treatment compliance.

PLAN

- Continue PT plan of care and medication optimization.
- Escalate to specialty consultation due to persistent deficits.


Dr. Emily Roberts

Office Follow-Up | Signed: 07/10/2023 16:20

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CONSULT NOTE - SPINE / NEUROLOGY

Date: 08/03/2023

Consult Team: Mercy Spine + Neurology

Referral Source: Dr. Emily Roberts

CONSULT FINDINGS

- Positive straight-leg raise on left with reproducible radicular pattern.
- Reduced left lower-extremity strength and diminished Achilles reflex documented serially.
- Clinical concern for progressive nerve-root compression despite non-operative treatment.

CONSULT RECOMMENDATION

- Proceed with lumbar MRI without contrast for anatomic clarification.
- Use MRI findings to determine next interventional or surgical step.


M. Chen, DO

Neurology Consultation | Signed: 08/03/2023 14:08

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PHYSICAL EXAMINATION TREND (SERIAL FINDINGS)

Date	Pain	SLR Left	LLE Strength	Achilles	Sensation	Sitting Tol.
06/20	8/10	35 deg	4-/5	Decreased	Calf/foot pares...	<15 min
07/10	7/10	35 deg	4-/5	Decreased	Persistent tingli...	<15 min
07/24	8/10	30 deg	4-/5	Decreased	Numbness worsen...	<12 min
08/03	8/10	28 deg	3+/5	Diminished	Dermatomal sens...	<10 min
08/07	8-9/10	25 deg	3+/5	Diminished	Persistent numb...	<10 min

REVIEWER NOTES

- Serial exams reflect progression of neurologic findings over time.

PT INITIAL EVALUATION - MERCY OUTPATIENT REHAB

Date:	06/26/2023
Diagnosis:	M54.16 / M51.26
Plan of Care:	06/26/2023 to 08/07/2023
Frequency:	2x/week

SUBJECTIVE

- 3-month history of low-back pain radiating to left posterior leg.
- Pain 8/10 with numbness/tingling in left calf and foot.
- Sitting tolerance under 15 minutes; sleep disrupted nightly.

OBJECTIVE

- SLR positive (left) at 35 degrees.
- Lumbar flexion limited 50%.
- Left LE strength 4-/5 with decreased Achilles reflex.
- ODI 52% (severe disability).

ASSESSMENT/PLAN

- Findings consistent with lumbar radiculopathy and significant functional limitation.
- Core stabilization, traction, neural mobilization, manual therapy, and HEP initiated.


Sarah Mitchell, PT, DPT

PT Initial Evaluation | Signed: 06/26/2023 11:40

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PT WEEKLY PROGRESS SUMMARY

WEEK 2 - 07/10/2023

- Pain 7/10 with persistent radicular symptoms.
- Slight ROM improvement but ODI still high at 48%.
- Functional improvement minimal.

WEEK 4 - 07/24/2023

- Pain returns to 8/10 with prolonged sitting.
- Continued numbness in left foot and SLR positive at 30 degrees.
- ODI worsened to 50% despite therapy compliance.


Sarah Mitchell, PT, DPT

PT Progress Note | Signed: 07/24/2023 17:02

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PT REASSESSMENT / DISCHARGE

Date: 08/07/2023

Total PT Visits: 12

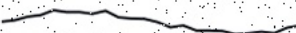
Home Program: Compliant

DISCHARGE FINDINGS

- Pain worsened to 8-9/10.
- SLR positive at 25 degrees with unchanged lumbar ROM limits.
- Left LE strength declined to 3+/5.
- ODI increased to 56% (worsened disability).

CLINICAL IMPRESSION

- Failed 6-week structured conservative management.
- Persistent radicular pain with progressive neurologic weakness.
- Further diagnostic imaging (lumbar MRI) recommended for next-step care.


Sarah Mitchell, PT, DPT

PT Discharge Summary | Signed: 08/07/2023 16:18

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PHYSICAL THERAPY VISIT LOG AND OUTCOMES

Visit	Date	Intervention	Pain	Outcome Summary
1	06/26	Initial evaluation + HEP	8/10	Severe disability; ODI 52%
2	06/29	Core stabilization / traction	8/10	Persistent left leg numbness
3	07/03	Neural mobilization	7/10	Minimal short-term relief
4	07/06	Manual therapy + HEP review	7/10	Sitting tolerance still limited
5	07/10	Progress reassessment	7/10	ODI 48%; minimal functional gain
6	07/13	Traction / progressive core	7/10	Radicular pain persists
7	07/17	Manual + neuromuscular re-ed	8/10	Pain rebound after activity
8	07/20	Neural glide progression	8/10	Left calf tingling ongoing
9	07/24	Week-4 reassessment	8/10	SLR worsened to 30 deg; ODI 50%
10	07/27	Lumbar stabilization	8/10	No sustained progress
11	08/01	HEP compliance check	8/10	Weakness persists despite adherence
12	08/07	Discharge reassessment	8-9/10	ODI 56%; progressive neurologic defi...

REVIEWER NOTES

- Visit-level outcomes show persistent high pain and deteriorating disability scores.

MEDICATION HISTORY

Medication	Dose	Start	Duration	Response / Outcome
Naproxen	500 mg BID	05/22/2023	Tried 6 weeks	Insufficient pain control
Gabapentin	300 mg TID	06/01/2023	Dose escalated	Persistent neuropathic pain
Cyclobenzaprine	10 mg HS	06/08/2023	Used PRN at night	Sleep disturbance persists
Methylpred dose pack	Taper	06/30/2023	Completed	Temporary mild benefit only
Tramadol	50 mg q8h PRN	07/21/2023	Intermittent res...	Pain rebounds with sitting
Topical lidocaine	5% patch	07/25/2023	Adjunctive	No meaningful functional gain

REVIEWER NOTES

- Medication escalation did not produce durable symptom or functional improvement.


Nina Patel, PharmD

Medication Review | Signed: 08/12/2023 10:36

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INJECTION AND PROCEDURE HISTORY

Date	Procedure	Provider	Immediate Response	Sustained Outcome
07/18/2023	L4-L5 transforaminal ESI	PM&R	Pain 8/10 -> 6/10 for...	Relief not durable
07/31/2023	Trigger-point lumbar paraspinal	Pain Clinic	Minimal transient res...	No functional change

REVIEWER NOTES

- Injection interventions yielded no durable relief and did not reverse neurologic decline.


R. Thompson, MD

Pain Procedure Review | Signed: 08/12/2023 11:02

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PRIOR IMAGING REPORT - LUMBAR X-RAY

Date:	05/22/2023
Facility:	Mercy Hospital Imaging
Study:	Lumbar Spine X-Ray, 3 views

FINDINGS

- Mild degenerative disc space narrowing at L4-L5.
- No acute fracture or malalignment.
- Radiographic findings non-specific for nerve compression.

IMPRESSION

- X-ray inconclusive for etiology of progressive radicular symptoms.
- Cross-sectional imaging recommended if neurologic deficits persist.


A. Simmons, MD

Radiology | Signed: 05/22/2023 15:47

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PRIOR IMAGING REPORT - LUMBAR CT

Date:	06/05/2023
Facility:	Mercy Hospital Imaging
Study:	CT Lumbar Spine without contrast

FINDINGS

- Left paracentral disc protrusion at L4-L5 with suspected contact of traversing L5 root.
- Mild central canal narrowing; no acute osseous injury.
- Limited soft tissue and nerve-root definition by CT modality.

IMPRESSION

- Findings concerning for disc pathology correlated with left-sided radiculopathy.
- MRI recommended for definitive nerve-root and soft tissue assessment.


A. Simmons, MD

Radiology | Signed: 06/05/2023 13:20

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HISTORICAL IMAGING REPORT - PRIOR LUMBAR MRI

Date: 03/19/2021
Facility: Mercy Hospital Imaging
Study: MRI Lumbar Spine without contrast

FINDINGS

- Mild broad-based L4-L5 disc bulge without focal nerve-root impingement at that time.
- No severe canal stenosis on 2021 study.

CLINICAL RELEVANCE

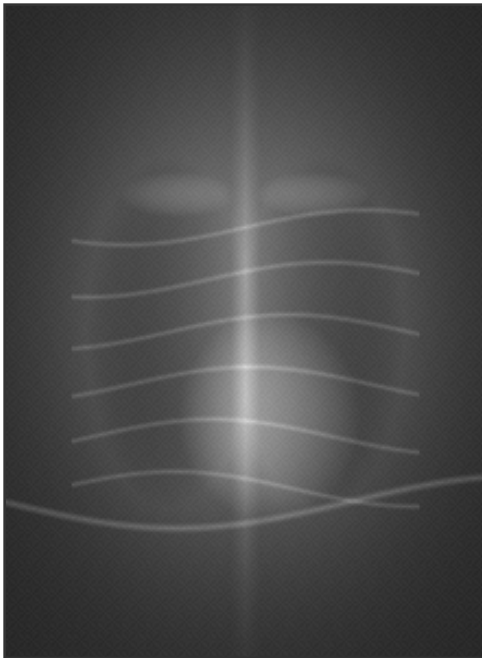
- Current 2023 symptoms and deficits have significantly progressed from baseline imaging.
- Updated MRI is needed to characterize new/progressive pathology.


J. Vega, MD

Radiology Archive Review | Signed: 08/11/2023 09:05

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SYNTHETIC IMAGING ATTACHMENT - LUMBAR CORRELATE 1



Interpretation

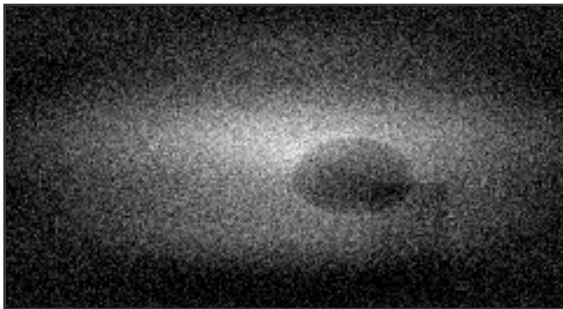
- Synthetic diagnostic attachment included to simulate a multi-modal imported record.
- Visual representation supports review workflow realism, not diagnostic use.
- Clinical interpretation remains in signed radiology reports.

Synthetic imaging placeholder for demo packet realism

SYNTHETIC IMAGING ATTACHMENT - LUMBAR CORRELATE 2

Interpretation

- Additional synthetic imaging page to reflect imported outside-document bundles.
- Used for demo of multi-modal ingestion within appeal packets.
- Does not replace formal radiologist interpretation.



Synthetic imaging placeholder for demo packet realism

CONSERVATIVE MANAGEMENT FAILURE SUMMARY

COMPLETED CONSERVATIVE INTERVENTIONS

- Supervised PT: 12 visits over 6 weeks with documented compliance.
- Medication trials: anti-inflammatory, neuropathic, muscle relaxant, rescue analgesic regimens.
- Injection procedures performed with only transient or absent benefit.

OUTCOME TREND

- Pain severity remained high (7-9/10).
- Neurologic deficits progressed on serial objective examinations.
- Functional metrics worsened (ODI trend deterioration, reduced sitting tolerance).


Dr. Emily Roberts

Clinical Summary | Signed: 08/14/2023 13:06

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MEDICAL NECESSITY DETERMINATION - LUMBAR MRI

RATIONALE

- Documented persistent radicular pain for >3 months with progressive neurologic deficits.
- Completed compliant conservative management including medications, PT, and injection therapy.
- Serial exams show worsening strength (4-/5 to 3+/5) and persistent reflex abnormality.
- PT outcomes worsened (ODI 52% to 56%) despite 12 supervised visits and home program adherence.
- Lumbar MRI is required to define nerve-root compression severity and guide definitive management.

REQUESTED SERVICE

- CPT 72148 MRI lumbar spine without contrast.
- Requested to evaluate compressive pathology and direct definitive treatment.


Dr. Emily Roberts

Medical Necessity Attestation | Signed: 08/15/2023 08:42

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PAYER CRITERIA CROSSWALK

Criterion	Evidence in Packet
Persistent severe radicular pain	Documented at office, consult, PT, and reassessment visits
Neurologic deficit progression	Strength decline + reflex change + dermatomal sensory findings
Failed 6+ weeks conservative treatment	12 PT visits, medication trials, injection history
Need for treatment planning	Neurology and spine consult recommend MRI for next-step intervention

REVIEWER NOTES

- Appeal evidence mapped to documented criteria to support overturn of MRI denial.

ATTENDING APPEAL ADDENDUM

ATTENDING STATEMENT

Given progressive weakness and failed conservative care, lumbar MRI remains medically necessary and time-sensitive.

FINAL REQUEST

- Reconsider and approve denied lumbar MRI request.
- Prevent further neurologic progression and prolonged disability.


Dr. Emily Roberts

Final Appeal Addendum | Signed: 08/15/2023 09:22

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